

CASE PRESENTATION PEDIATRICS

By
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Demographic information:

Name: Master XYZ

Age:3years 2months

Sex: Male

Address:Hulkoti, Gadag

Religion:Hindu

Informant:Mother: ABC Age:26yrs Education:10th std

DOA:3/3/2020

DOE:4/3/2020

Chief complaints:

- Cough since 6days
- Fever since 5days
- Hurried breathing since 3days

History of presenting illness:

- The child was apparently well 6 days back then developed cough which was insidious in onset . There are no aggravating factors. Cough has relieved after taking medication. There is no diurnal variations. There is also history of rapid respiration, noisy breathing.
- There is also history of fever since 5 days. Which is sudden in onset, present throughout the day, high grade fever, associated with chills, not associated with rashes, relieved after medications.

- There is also history of hurried breathing since 3days. It is of insidious in onset , progressive in nature. There is also h/o chest retractions, grunting.
- There is history of irritability, excessive crying, decreased food intake since a week.
- No h/o running nose,ear pain or ear discharge.
- No h/o mouth breathing, snoring, difficulty in swallowing, hoarseness of voice.
- No h/o bluishish discoloration of lips or extremities
- No h/o vomiting, diarrhoea.

- No h/o contact with TB patient, loss of appetite
- No h/o chest pain
- No h/o wheeze
- No h/o day time sleepiness, altered sensorium, convulsions
- No h/o bleeding from nose, trauma to the chest.
- No h/o progressive pallor, pica, bitots spot, bleeding gums.
- No h/o similar complaints in the sibling or any family members.

Past History:

- There is similar history in the past when he was 2 yrs. The child was admitted to hospital for a week with complaints of cough, fever, difficulty in breathing. and was cured completely.
- There is no h/o bronchial asthma, or other illnesses.

Treatment History:

- IV fluids, paracetamol, cotrimoxazole are been given.
- Now the child has improved.

Birth history:

- **Antenatal history:**
- Booked case
- Birth space -2 yrs
- Weight gain in pregnancy - 8 kgs
- Anamoly scan was done
- Iron and folic acid tablets were taken
- 2 injections of tetanus toxoid taken
- No h/o fever
- No h/ o diabetes mellitus, hypertension, or other chronic illnesses

- **Natal history:**
- Full term, normal vaginal delivery
- Birth weight- 3kg, cried immediately after birth
- No h/o NICU admissions
- No h/o any congenital anomalies
- Breastfeeding started after half an hour of delivery. No pretactéal feeds were given.
- **Postnatal history:**
- Child was exclusively breastfed for 6 months and weaning was started after 6 months.
- No h/o hospitalisation

Development history:

- **Gross motor:** rides tricycle, alternate feet going upstairs
- **Fine motor:** copies circle, builds tower of 9 blocks
- **Personal and social development:** knows his full name and gender
- **Language:** ask questions, tells it's name
- Developmental milestones are achieved appropriately and it corresponds to chronological age.

Immunisation history:

- At birth BCG, opv, hep b taken
- At 6 weeks: opv 1, pentavalent 1st dose taken
- At 10 weeks: opv2, pentavalent 2nd dose taken
- At 14 weeks: opv3, pentavalent 3rd dose taken
- At 9 months: MMR
- At 16-24 months: DPT booster taken, OPV booster
- Child is fully immunised upto date.

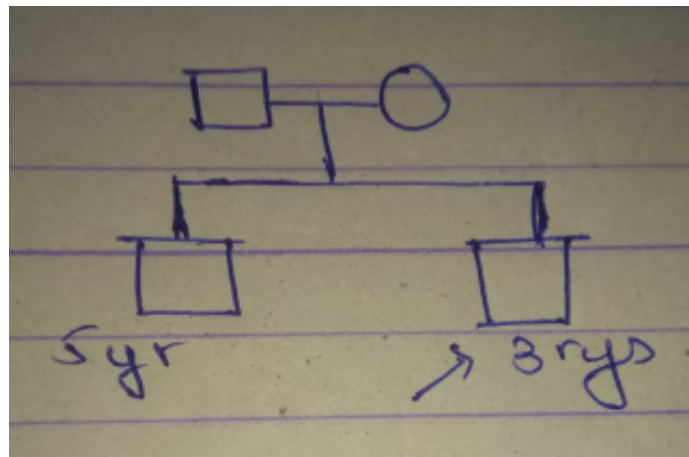
Nutritional history:

- 24hr recall method-
- Morning- 1chapati+ 1cup bhaji - 385 kcal, 5gm
- Afternoon- rice+ sambar - 355kcal, 3g
- Evening- 1cup milk-40kcal, 2g
- Dinner- rice+ sambar- 355kcal,3g

- Total- 1135kcal, 13g
- Required- 1400kcal, 15.4 g
- Deficient-265 kcal, 2.4 g
- Percentage-19%in calories, 15.58% in proteins

Family history:

- No h/o genetic or chromosomal disorders in the family
- No h/o bronchial asthma in the family
- Type of family- joint family (10 members)
- No contact with TB patient
- Married life - 6yrs, non-consanguineous marriage



Socioeconomic status:

- Father is a farmer, studied till 10th std
- Belongs to class 3 according to BG Prasad classification
- Lives in a pucca house
- No h/o firewood practices , smoking in family members
- Overcrowding is present, bad ventilation
- Open defecation
- Municipal water supply stored in closed container

Summary:

- Here is a 3years 2 months old male child born out of non consanguineous marriage, came with history of cough since 6 days, fever since 5 days and difficulty in breathing since 3 days. There is h/o grunting, chest retractions, refusal of feeding. There is similar history one year back. The child belonging to class 3 according to BG Prasad classification, who is completely immunised, with normal developmental milestones, with diet which is 19% deficient in calories and 15.58 % deficient in proteins.
- So according to the history it may be a case of lower respiratory tract infection.

Examination:

- General Survey:
- Child is irritable, alert, moves around
- Examined in sitting posture
- Vital signs:
- Temperature: 100.4 degree Fahrenheit
- Respiratory rate: 60 cycles per minute, abdominothoracic, flaring of ala nasi, use of accessory muscles seen.
- Pulse: 120 beats per minute, normal in rhythm, volume, character, measured on right radial artery, no radio-radial delay or radio femoral delay.
- Blood pressure: 100/80mm hg measured over right brachial artery in supine position

Head to toe examination:

- Skull shape: normal, fontanales closed
- Hair: black , no colour changes
- Eyes: no pallor, icterus, kf rings, bitot's spot, dryness
- Ears: no discharge seen
- Nose: flaring of ala nasai seen , no nasal deviations
- Lips: no cleft lip, no cyanosis
- Mouth: no cheliosis or stomatitis, bleeding gums

- Tongue: no cyanosis, appears to be normal
- Palate: no high arched palate or cleft palate
- Teeth: normal dentition present
- Neck: normal
- Nails: no clubbing
- Skin: no skin lesions seen
- Abdomen and genitalia: normal
- Spine, and back: normal
- Lower limbs : normal
- No edema or lymphadenopathy

Anthropometry:

- Weight- 14 kg within 50th percentile
- Height- 100cm within 97th percentile
- HC- 50cm within 97th percentile
- MAC- 15 cms
- CC- 60cms

Systemic examination:

- **Respiratory system examination:**
- **UPPER RESPIRATORY TRACT:**
- Nose: flaring of nasal ala seen, nasal septum is normal
- Air sinuses: no tenderness of frontal or maxillary sinuses felt
- Oropharynx: appears to be normal, there is no enlargement of tonsils
- Pharynx: posterior wall appears to be congested

LOWER RESPIRATORY TRACT:

- **Inspection:**
- Shape of chest : elliptical in shape
- Movements with respiration: decreased on right side
- Trachea appears to be in center
- Both nipples are at same level
- Apical beat is not seen
- Subcostal and intercostal retractions seen and there is use of accessory muscles.
- No scars, sinuses, dilated veins seen
- No dropping of shoulder, crowding of ribs

- **Palpation:**
- All inspectory findings confirmed
- No tenderness
- No rise in local temperature
- Position of trachea- central
- Apical impulse- felt at fifth intercoastal space in midclavicular line
- Chest expansion- decreased on right side
- Vocal fremitus: could not be elicited

Percussion:

Areas	Rt	Lt
Supraclavicular	R	R
Infraclavicular	R	R
Mammary	Dullness	R
Axillary	R	R
Infraaxillary	Dullness	R
Suprascapular	R	R
Infrascapular	Dullness	R
Interscapular	R	R

Auscultation:

Breathsounds

Areas	Rt	Lt
Supraclavicular	V	V
Infraclavicular	V	V
Mammary	B	V
Axillary	V	V
Infraaxillary	B	V
Suprascapular	V	V
Infrascapular	B	V
Interscapular	V	V

- Vocal resonance: could not be elicited.
- No added sounds heard.

Other systems:

- **CVS:**
- No raised jvp, S1 S2 heard
- **PER ABDOMEN:**
- No hepatomegaly or splenomegaly
- **CNS:**
- Higher mental functions are normal, no sensory or motor deficit

Summary:

- Here is a 3years 2 months old male child born out of non consanguineous marriage. Presented with complaints of cough since 6days, fever since 5days , difficulty in breathing since 3days.
- O/E there was fever, no pallor, cyanosis, clubbing.
- O/E of respiratory system, chest expansion decreased on right side, chest retractions are seen, trachea is central, dullness felt on percussion in right mammary, infraaxillary and infrascapular areas and also on auscultation bronchial breath sounds heard on right mammary, infraaxillary, infrascapular areas.

Provisional Diagnosis:

- It is a case of lower respiratory tract infection most probably Very Severe Pneumonia.

Investigations:

- CBC
- ESR
- CRP
- Chest X-ray
- Throat swab for culture sensitivity
- ABG
- Serum electrolytes

THANK YOU